

INPATIENT INSULIN INFUSION TARGETS

The synthetic guidance suggests a glucose target range of one hundred forty to one hundred eighty milligrams per deciliter for most non-critically-ill hospitalized adults receiving insulin. A tighter target of one hundred ten to one hundred forty milligrams per deciliter may be appropriate for selected critically-ill patients if it can be achieved without significant hypoglycemia.

Continuous insulin infusion is suggested for severe hyperglycemia, diabetic ketoacidosis, and the perioperative period in cardiac surgery, with hourly bedside glucose monitoring during titration.

BASAL BOLUS REGIMEN

For non-critically-ill inpatients, the synthetic recommendation favors a basal-bolus regimen over sliding-scale-only insulin. A typical total daily dose starts at 0.3 to 0.5 units per kilogram, divided as fifty percent basal and fifty percent prandial when oral intake is reliable. Correction insulin is added on top for pre-meal hyperglycemia.

HYPOGLYCEMIA MANAGEMENT

Treatment of glucose below seventy milligrams per deciliter is suggested with fifteen grams of fast-acting carbohydrate orally if the patient can swallow safely, or twenty-five grams of dextrose intravenously otherwise, with re-check in fifteen minutes. Insulin regimen should be reviewed and adjusted after any episode below fifty-four milligrams per deciliter.